ICSR Narrative Report

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Source data	icsr_source_cases.csv;
Reference safety information	Synthetic Core Data Sheet v1.0 (listedness taken from the supplied reference only)
Case type / source	Spontaneous reports; country, receipt date and sender not supplied
MedDRA version used	v29.1
Causality method	WHO-UMC categories;

1. Executive summary

Twelve cases were assessed: 10 serious (8 medically important, 2 hospitalisation) and 2 non-serious. All 12 meet the patient, suspect-product and adverse-event criteria for a valid case. The reporter criterion is only partly met (qualification given; no name or initials, contact details or country), so a reporter follow-up is required before expedited submission.

Eleven of the 12 events are listed in the supplied reference. The single unlisted pair is **losartan - fatigue (SYN-PV-008)**; it is non-serious, so no expedited report is needed, but it should be reviewed in aggregate monitoring.

A provisional WHO-UMC causality assessment is given for each case: Probable/Likely for 006, 009, 011 and 012; Possible for 001, 004, 005, 008 and 010; Unlikely for 002, 003 and 007. Key concerns: 10 of 12 drug/indication pairs are clinically implausible; doses in 004 and 007 are implausible; dechallenge/rechallenge entries are inconsistent in several cases; and no dates were supplied, so reporting clocks cannot be calculated.

2. Case validity (minimum criteria, all 12 cases)

Criterion	Evidence in source	Assessment	Action
Identifiable patient	Age and (where known) sex supplied for every case	**Met.** One identifier such as age, age group or sex is sufficient (ICH E2D / GVP Module VI).	None
Identifiable reporter	Qualification only (consumer, pharmacist, physician); no name/initials, contact or country	**Partly met.** Valid for processing, but Day 0 and submission depend on a traceable reporter and country (required in E2B(R3)).	Send reporter follow-up
Suspect product	Product, dose, route and indication supplied	**Met.** Product is named; dose and indication need verification.	Verify flagged values
Adverse event	One reaction term per case	**Met.**	Obtain clinical description

Conclusion: all 12 cases are valid for processing; the reporter follow-up is outstanding.

3. MedDRA term mapping

Each verbatim was matched to the LLT of the same wording, then to its PT and primary SOC. “Peripheral oedema” is an LLT; its PT is “Oedema peripheral”.

Case	Verbatim	LLT	PT	PT code	Primary SOC
SYN-PV-001	Rash	Rash	Rash	10037844	Skin and subcutaneous tissue disorders

SYN-PV-002	Myalgia	Myalgia	Myalgia	10028411	Musculoskeletal and connective tissue disorders
SYN-PV-003	Nausea	Nausea	Nausea	10028813	Gastrointestinal disorders
SYN-PV-004	Peripheral oedema	Peripheral oedema	Oedema peripheral	10030124	General disorders and administration site conditions
SYN-PV-005	Tendon pain	Tendon pain	Tendon pain	10066371	Musculoskeletal and connective tissue disorders
SYN-PV-006	Dizziness	Dizziness	Dizziness	10013573	Nervous system disorders
SYN-PV-007	Headache	Headache	Headache	10019211	Nervous system disorders
SYN-PV-008	Fatigue	Fatigue	Fatigue	10016256	General disorders and administration site conditions
SYN-PV-009	Rash	Rash	Rash	10037844	Skin and subcutaneous tissue disorders
SYN-PV-010	Myalgia	Myalgia	Myalgia	10028411	Musculoskeletal and connective tissue disorders
SYN-PV-011	Nausea	Nausea	Nausea	10028813	Gastrointestinal disorders
SYN-PV-012	Peripheral oedema	Peripheral oedema	Oedema peripheral	10030124	General disorders and administration site conditions

4. Expectedness (listedness) against the supplied reference

Case	Product	Reaction (PT)	Reference status	Listedness
SYN-PV-001	Amoxicillin	Rash	Known	Listed
SYN-PV-002	Atorvastatin	Myalgia	Known	Listed
SYN-PV-003	Metformin	Nausea	Known	Listed
SYN-PV-004	Amlodipine	Oedema peripheral	Known	Listed
SYN-PV-005	Ciprofloxacin	Tendon pain	Known	Listed
SYN-PV-006	Sertraline	Dizziness	Known	Listed
SYN-PV-007	Omeprazole	Headache	Known	Listed
SYN-PV-008	Losartan	Fatigue	Unexpected	Unlisted
SYN-PV-009	Amoxicillin	Rash	Known	Listed
SYN-PV-010	Atorvastatin	Myalgia	Known	Listed
SYN-PV-011	Metformin	Nausea	Known	Listed
SYN-PV-012	Amlodipine	Oedema peripheral	Known	Listed

5. Individual case narratives

SYN-PV-001: Amoxicillin / Rash

Patient	30 years; sex: Male. Name/initials, DOB, weight, height, country: not reported.
Reporter	Consumer. Name, contact details, country: not reported.
Suspect drug	Amoxicillin; 20 mg once daily; oral; indication: Anxiety (as reported). Batch and start/stop dates: not reported.
Concomitant	None reported. Medical history/allergies: not reported.
Reaction	Verbatim: Rash. LLT: Rash; PT: Rash; SOC: Skin and subcutaneous tissue disorders.
Onset / outcome	17 day(s) after start of treatment; outcome: Not recovered. Exact dates: not reported.
Seriousness	Serious - other medically important condition (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Negative; rechallenge: Positive.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Possible (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a consumer and concerns a 30-year-old male who was treated with amoxicillin 20 mg once daily, oral, for a reported indication of anxiety (dose and indication as reported; verification requested). Treatment start and stop dates were not provided. Approximately 17 days after treatment started, the patient experienced rash (MedDRA PT: Rash). The reporter/source classified the event as serious (medically important); the specific rationale was not provided. Treatment with amoxicillin was stopped (action taken: drug withdrawn, per source narrative); event did not abate after withdrawal (negative dechallenge), and the event recurred on re-exposure (positive rechallenge). At last report the patient had not recovered. No concomitant medication was reported. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** possible. Conflicting dechallenge (negative) and rechallenge (positive); delayed onset (17 days) is compatible with delayed hypersensitivity to a beta-lactam; no concomitant drugs. Cannot be upgraded until re-exposure details are obtained.

SYN-PV-002: Atorvastatin / Myalgia

Patient	36 years; sex: Unknown. Name/initials, DOB, weight, height, country: not reported.
Reporter	Pharmacist. Name, contact details, country: not reported.
Suspect drug	Atorvastatin; 5 mg once daily; oral; indication: Anxiety (as reported). Batch and start/stop dates: not reported.
Concomitant	None reported. Medical history/allergies: not reported.
Reaction	Verbatim: Myalgia. LLT: Myalgia; PT: Myalgia; SOC: Musculoskeletal and connective tissue disorders.
Onset / outcome	1 day(s) after start of treatment; outcome: Recovered. Exact dates: not reported.
Seriousness	Non-serious (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Unknown; rechallenge: Negative.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Unlikely (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a pharmacist and concerns a 36-year-old patient of unknown sex who was treated with atorvastatin 5 mg once daily, oral, for a reported indication of anxiety (dose and indication as reported; verification requested). Treatment start and

stop dates were not provided. Approximately 1 day after treatment started, the patient experienced myalgia (MedDRA PT: Myalgia). The event was classified as non-serious. Treatment with atorvastatin was stopped (action taken: drug withdrawn, per source narrative); dechallenge result unknown, and the event did not recur on re-exposure (negative rechallenge). At last report the patient had recovered. No concomitant medication was reported. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** unlikely. Onset on day 1 is atypically short for statin-associated myalgia and the event did not recur on rechallenge; dechallenge unknown.

SYN-PV-003: Metformin / Nausea

Patient	34 years; sex: Female. Name/initials, DOB, weight, height, country: not reported.
Reporter	Consumer. Name, contact details, country: not reported.
Suspect drug	Metformin; 5 mg once daily; oral; indication: Anxiety (as reported). Batch and start/stop dates: not reported.
Concomitant	Vitamin D. Medical history/allergies: not reported.
Reaction	Verbatim: Nausea. LLT: Nausea; PT: Nausea; SOC: Gastrointestinal disorders.
Onset / outcome	13 day(s) after start of treatment; outcome: Recovered. Exact dates: not reported.
Seriousness	Serious - caused/prolonged hospitalisation (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Negative; rechallenge: Negative.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Unlikely (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a consumer and concerns a 34-year-old female who was treated with metformin 5 mg once daily, oral, for a reported indication of anxiety (dose and indication as reported; verification requested). Treatment start and stop dates were not provided. Approximately 13 days after treatment started, the patient experienced nausea (MedDRA PT: Nausea). The event was classified as serious (hospitalisation); admission and discharge dates and the reason for admission were not provided. Treatment with metformin was stopped (action taken: drug withdrawn, per source narrative); event did not abate after withdrawal (negative dechallenge), and the event did not recur on re-exposure (negative rechallenge). At last report the patient had recovered. Concomitant medication: vitamin D. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** unlikely. Event listed and timing plausible, but it did not abate on withdrawal and did not recur on re-exposure (both negative). Hospitalisation for nausea alone needs clinical clarification.

SYN-PV-004: Amlodipine / Oedema peripheral

Patient	61 years; sex: Male. Name/initials, DOB, weight, height, country: not reported.
Reporter	Pharmacist. Name, contact details, country: not reported.
Suspect drug	Amlodipine; 500 mg twice daily; oral; indication: Anxiety (as reported). Batch and start/stop dates: not reported.
Concomitant	None reported. Medical history/allergies: not reported.
Reaction	Verbatim: Peripheral oedema. LLT: Peripheral oedema; PT: Oedema peripheral; SOC: General disorders and administration site conditions.

Onset / outcome	21 day(s) after start of treatment; outcome: Recovered. Exact dates: not reported.
Seriousness	Serious - other medically important condition (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Positive; rechallenge: Negative.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Possible (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a pharmacist and concerns a 61-year-old male who was treated with amlodipine 500 mg twice daily, oral, for a reported indication of anxiety (dose and indication as reported; verification requested). Treatment start and stop dates were not provided. Approximately 21 days after treatment started, the patient experienced peripheral oedema (MedDRA PT: Oedema peripheral). The reporter/source classified the event as serious (medically important); the specific rationale was not provided. Treatment with amlodipine was stopped (action taken: drug withdrawn, per source narrative); event abated after withdrawal (positive dechallenge), and the event did not recur on re-exposure (negative rechallenge). At last report the patient had recovered. No concomitant medication was reported. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** possible. Listed, dose-related effect with positive dechallenge, but rechallenge negative. The reported dose (1,000 mg/day) is about 100-fold above the maximum labelled dose: possible overdose or transcription/unit error.

SYN-PV-005: Ciprofloxacin / Tendon pain

Patient	33 years; sex: Unknown. Name/initials, DOB, weight, height, country: not reported.
Reporter	Pharmacist. Name, contact details, country: not reported.
Suspect drug	Ciprofloxacin; 500 mg twice daily; oral; indication: Infection (as reported). Batch and start/stop dates: not reported.
Concomitant	None reported. Medical history/allergies: not reported.
Reaction	Verbatim: Tendon pain. LLT: Tendon pain; PT: Tendon pain; SOC: Musculoskeletal and connective tissue disorders.
Onset / outcome	15 day(s) after start of treatment; outcome: Not recovered. Exact dates: not reported.
Seriousness	Serious - other medically important condition (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Unknown; rechallenge: Not done.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Possible (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a pharmacist and concerns a 33-year-old patient of unknown sex who was treated with ciprofloxacin 500 mg twice daily, oral, for a reported indication of infection (dose and indication as reported; verification requested). Treatment start and stop dates were not provided. Approximately 15 days after treatment started, the patient experienced tendon pain (MedDRA PT: Tendon pain). The reporter/source classified the event as serious (medically important); the specific rationale was not provided. Treatment with ciprofloxacin was stopped (action taken: drug withdrawn, per source narrative); dechallenge result unknown, and no rechallenge was performed. At last report the patient had not recovered. No concomitant medication was reported. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0.

Causality (company, WHO-UMC): possible. Fluoroquinolone-class effect on tendons; timing (15 days) plausible; no concomitant drugs. Dechallenge unknown, rechallenge not done, event ongoing; underlying infection is a possible confounder.

SYN-PV-006: Sertraline / Dizziness

Patient	31 years; sex: Unknown. Name/initials, DOB, weight, height, country: not reported.
Reporter	Physician. Name, contact details, country: not reported.
Suspect drug	Sertraline; 20 mg once daily; oral; indication: Hypertension (as reported). Batch and start/stop dates: not reported.
Concomitant	Paracetamol. Medical history/allergies: not reported.
Reaction	Verbatim: Dizziness. LLT: Dizziness; PT: Dizziness; SOC: Nervous system disorders.
Onset / outcome	3 day(s) after start of treatment; outcome: Recovering. Exact dates: not reported.
Seriousness	Serious - other medically important condition (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Positive; rechallenge: Not done.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Probable/Likely (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a physician and concerns a 31-year-old patient of unknown sex who was treated with sertraline 20 mg once daily, oral, for a reported indication of hypertension (dose and indication as reported; verification requested). Treatment start and stop dates were not provided. Approximately 3 days after treatment started, the patient experienced dizziness (MedDRA PT: Dizziness). The reporter/source classified the event as serious (medically important); the specific rationale was not provided. Treatment with sertraline was stopped (action taken: drug withdrawn, per source narrative); event abated after withdrawal (positive dechallenge), and no rechallenge was performed. At last report the patient was recovering. Concomitant medication: paracetamol. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** probable/likely. Plausible short latency, listed event, positive dechallenge; paracetamol unlikely cause. Confounders: reported hypertension and indication mismatch (sertraline is not used for hypertension).

SYN-PV-007: Omeprazole / Headache

Patient	63 years; sex: Male. Name/initials, DOB, weight, height, country: not reported.
Reporter	Consumer. Name, contact details, country: not reported.
Suspect drug	Omeprazole; 500 mg twice daily; oral; indication: Hypertension (as reported). Batch and start/stop dates: not reported.
Concomitant	Paracetamol. Medical history/allergies: not reported.
Reaction	Verbatim: Headache. LLT: Headache; PT: Headache; SOC: Nervous system disorders.
Onset / outcome	16 day(s) after start of treatment; outcome: Recovered. Exact dates: not reported.
Seriousness	Serious - other medically important condition (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Negative; rechallenge: Negative.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Unlikely (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a consumer and concerns a 63-year-old male who was treated with omeprazole 500 mg twice daily, oral, for a reported indication of

hypertension (dose and indication as reported; verification requested). Treatment start and stop dates were not provided. Approximately 16 days after treatment started, the patient experienced headache (MedDRA PT: Headache). The reporter/source classified the event as serious (medically important); the specific rationale was not provided. Treatment with omeprazole was stopped (action taken: drug withdrawn, per source narrative); event did not abate after withdrawal (negative dechallenge), and the event did not recur on re-exposure (negative rechallenge). At last report the patient had recovered. Concomitant medication: paracetamol. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** unlikely. Dechallenge and rechallenge both negative; reported dose (1,000 mg/day) is far above the usual 20-40 mg/day; underlying hypertension is a plausible alternative cause.

SYN-PV-008: Losartan / Fatigue

Patient	36 years; sex: Male. Name/initials, DOB, weight, height, country: not reported.
Reporter	Pharmacist. Name, contact details, country: not reported.
Suspect drug	Losartan; 5 mg once daily; oral; indication: Infection (as reported). Batch and start/stop dates: not reported.
Concomitant	Paracetamol. Medical history/allergies: not reported.
Reaction	Verbatim: Fatigue. LLT: Fatigue; PT: Fatigue; SOC: General disorders and administration site conditions.
Onset / outcome	2 day(s) after start of treatment; outcome: Recovering. Exact dates: not reported.
Seriousness	Non-serious (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Negative; rechallenge: Not done.
Listedness	Unlisted (Synthetic CDS v1.0).
Causality	Possible (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a pharmacist and concerns a 36-year-old male who was treated with losartan 5 mg once daily, oral, for a reported indication of infection (dose and indication as reported; verification requested). Treatment start and stop dates were not provided. Approximately 2 days after treatment started, the patient experienced fatigue (MedDRA PT: Fatigue). The event was classified as non-serious. Treatment with losartan was stopped (action taken: drug withdrawn, per source narrative); event did not abate after withdrawal (negative dechallenge), and no rechallenge was performed. At last report the patient was recovering. Concomitant medication: paracetamol. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is classified as **unlisted (unexpected)** against the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** possible. Temporal association present, but negative dechallenge; the infection being treated is an obvious alternative cause of fatigue. Event unlisted in the supplied reference.

SYN-PV-009: Amoxicillin / Rash

Patient	78 years; sex: Unknown. Name/initials, DOB, weight, height, country: not reported.
Reporter	Physician. Name, contact details, country: not reported.
Suspect drug	Amoxicillin; 5 mg once daily; oral; indication: Anxiety (as reported). Batch and start/stop dates: not reported.
Concomitant	Vitamin D. Medical history/allergies: not reported.
Reaction	Verbatim: Rash. LLT: Rash; PT: Rash; SOC: Skin and subcutaneous tissue disorders.

Onset / outcome	7 day(s) after start of treatment; outcome: Recovering. Exact dates: not reported.
Seriousness	Serious - other medically important condition (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Positive; rechallenge: Not done.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Probable/Likely (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a physician and concerns a 78-year-old patient of unknown sex who was treated with amoxicillin 5 mg once daily, oral, for a reported indication of anxiety (dose and indication as reported; verification requested). Treatment start and stop dates were not provided. Approximately 7 days after treatment started, the patient experienced rash (MedDRA PT: Rash). The reporter/source classified the event as serious (medically important); the specific rationale was not provided. Treatment with amoxicillin was stopped (action taken: drug withdrawn, per source narrative); event abated after withdrawal (positive dechallenge), and no rechallenge was performed. At last report the patient was recovering. Concomitant medication: vitamin D. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** probable/likely. Plausible timing, listed event, positive dechallenge with recovery in progress; vitamin D is an unlikely cause.

SYN-PV-010: Atorvastatin / Myalgia

Patient	57 years; sex: Female. Name/initials, DOB, weight, height, country: not reported.
Reporter	Pharmacist. Name, contact details, country: not reported.
Suspect drug	Atorvastatin; 5 mg once daily; oral; indication: Infection (as reported). Batch and start/stop dates: not reported.
Concomitant	Paracetamol. Medical history/allergies: not reported.
Reaction	Verbatim: Myalgia. LLT: Myalgia; PT: Myalgia; SOC: Musculoskeletal and connective tissue disorders.
Onset / outcome	21 day(s) after start of treatment; outcome: Recovered. Exact dates: not reported.
Seriousness	Serious - caused/prolonged hospitalisation (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Unknown; rechallenge: Negative.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Possible (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a pharmacist and concerns a 57-year-old female who was treated with atorvastatin 5 mg once daily, oral, for a reported indication of infection (dose and indication as reported; verification requested). Treatment start and stop dates were not provided. Approximately 21 days after treatment started, the patient experienced myalgia (MedDRA PT: Myalgia). The event was classified as serious (hospitalisation); admission and discharge dates and the reason for admission were not provided. Treatment with atorvastatin was stopped (action taken: drug withdrawn, per source narrative); dechallenge result unknown, and the event did not recur on re-exposure (negative rechallenge). At last report, the patient had recovered. Concomitant medication: paracetamol. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** possible. Listed event with plausible timing; rechallenge negative argues against but re-exposure conditions are unknown; infection and paracetamol are

possible confounders. Hospitalisation for myalgia requires clarification (e.g. CK to exclude rhabdomyolysis; no diagnosis is inferred).

SYN-PV-011: Metformin / Nausea

Patient	23 years; sex: Unknown. Name/initials, DOB, weight, height, country: not reported.
Reporter	Consumer. Name, contact details, country: not reported.
Suspect drug	Metformin; 5 mg once daily; oral; indication: Hypertension (as reported). Batch and start/stop dates: not reported.
Concomitant	None reported. Medical history/allergies: not reported.
Reaction	Verbatim: Nausea. LLT: Nausea; PT: Nausea; SOC: Gastrointestinal disorders.
Onset / outcome	19 day(s) after start of treatment; outcome: Recovering. Exact dates: not reported.
Seriousness	Serious - other medically important condition (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Unknown; rechallenge: Positive.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Probable/Likely (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a consumer and concerns a 23-year-old patient of unknown sex who was treated with metformin 5 mg once daily, oral, for a reported indication of hypertension (dose and indication as reported; verification requested). Treatment start and stop dates were not provided. Approximately 19 days after treatment started, the patient experienced nausea (MedDRA PT: Nausea). The reporter/source classified the event as serious (medically important); the specific rationale was not provided. Treatment with metformin was stopped (action taken: drug withdrawn, per source narrative); dechallenge result unknown, and the event recurred on re-exposure (positive rechallenge). At last report the patient was recovering. No concomitant medication was reported. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** probable/likely. Listed event, plausible timing and recurrence on re-exposure (positive rechallenge); dechallenge unknown, so not "Certain".

SYN-PV-012: Amlodipine / Oedema peripheral

Patient	24 years; sex: Female. Name/initials, DOB, weight, height, country: not reported.
Reporter	Pharmacist. Name, contact details, country: not reported.
Suspect drug	Amlodipine; 5 mg once daily; oral; indication: Hypertension (as reported). Batch and start/stop dates: not reported.
Concomitant	None reported. Medical history/allergies: not reported.
Reaction	Verbatim: Peripheral oedema. LLT: Peripheral oedema; PT: Oedema peripheral; SOC: General disorders and administration site conditions.
Onset / outcome	4 day(s) after start of treatment; outcome: Not recovered. Exact dates: not reported.
Seriousness	Serious - other medically important condition (source classification, retained).
Action / dech / rech	Drug withdrawn (per source narrative); dechallenge: Unknown; rechallenge: Positive.
Listedness	Listed (Synthetic CDS v1.0).
Causality	Probable/Likely (WHO-UMC, company, provisional)

Narrative. This spontaneous report was received from a pharmacist and concerns a 24-year-old female who was treated with amlodipine 5 mg once daily, oral, for a reported indication of hypertension (dose and indication as reported; verification requested). Treatment start and stop

dates were not provided. Approximately 4 days after treatment started, the patient experienced peripheral oedema (MedDRA PT: Oedema peripheral). The reporter/source classified the event as serious (medically important); the specific rationale was not provided. Treatment with amlodipine was stopped (action taken: drug withdrawn, per source narrative); dechallenge result unknown, and the event recurred on re-exposure (positive rechallenge). At last report the patient had not recovered. No concomitant medication was reported. Medical history, allergies, laboratory/diagnostic data and treatment of the event were not reported. The event is listed in the Synthetic Core Data Sheet v1.0. **Causality (company, WHO-UMC):** probable/likely. Listed dose-related effect, appropriate indication and dose, no concomitant drugs, recurrence on re-exposure. Outcome not recovered; dechallenge unknown.

6. Seriousness and reportability

Case	Source seriousness	E2B criterion	Listed?	Expedited?
SYN-PV-001	Medically important	other medically important condition	Yes	Yes
SYN-PV-002	Not serious	Non-serious	Yes	No (periodic; signal review)
SYN-PV-003	Hospitalisation	caused/prolonged hospitalisation	Yes	Yes
SYN-PV-004	Medically important	other medically important condition	Yes	Yes
SYN-PV-005	Medically important	other medically important condition	Yes	Yes
SYN-PV-006	Medically important	other medically important condition	Yes	Yes
SYN-PV-007	Medically important	other medically important condition	Yes	Yes
SYN-PV-008	Not serious	Non-serious	No	No (periodic; signal review)
SYN-PV-009	Medically important	other medically important condition	Yes	Yes
SYN-PV-010	Hospitalisation	caused/prolonged hospitalisation	Yes	Yes
SYN-PV-011	Medically important	other medically important condition	Yes	Yes
SYN-PV-012	Medically important	other medically important condition	Yes	Yes

The reporter's seriousness classification is retained and has not been downgraded. No supporting clinical rationale was supplied, and the rationale for "medically important" on common events (rash, headache, dizziness, oedema) must be requested. Serious cases (001, 003, 004, 005, 006, 007, 009, 010, 011, 012) are subject to expedited reporting (typically 15 calendar days from Day 0 under EU/ICH practice; in the US, serious and unexpected). Day 0 and due dates cannot be calculated because the receipt date and country are not supplied. Cases 002 and 008 are non-expedited; 008 (unlisted) should be reviewed for signal detection and included in periodic reports (PBRER/PSUR).

7. Dechallenge and rechallenge review

Case	Dechallenge	Rechallenge	Outcome	Observation
SYN-PV-001	Negative	Positive	Not recovered	Conflicting entries; positive rechallenge needs re-exposure details.
SYN-PV-002	Unknown	Negative	Recovered	Rechallenge recorded with dechallenge unknown.
SYN-PV-003	Negative	Negative	Recovered	Negative dechallenge but recovered; clarify.
SYN-PV-004	Positive	Negative	Recovered	Positive dechallenge, negative rechallenge: conflicting.
SYN-PV-005	Unknown	Not done	Not recovered	Rechallenge not done; not a negative rechallenge.
SYN-PV-006	Positive	Not done	Recovering	Consistent with a drug effect; confirm timing.
SYN-PV-007	Negative	Negative	Recovered	Negative dechallenge but recovered; clarify.
SYN-PV-008	Negative	Not done	Recovering	Negative dechallenge but recovering; clarify.

SYN-PV-009	Positive	Not done	Recovering	Consistent with a drug effect; confirm timing.
SYN-PV-010	Unknown	Negative	Recovered	Rechallenge recorded with dechallenge unknown.
SYN-PV-011	Unknown	Positive	Recovering	Rechallenge recorded with dechallenge unknown.
SYN-PV-012	Unknown	Positive	Not recovered	Rechallenge recorded with dechallenge unknown.

A positive dechallenge or rechallenge supports but does not establish causality. "Not done", "negative" and "unknown" are distinct values. A rechallenge implies the drug was first stopped, so rechallenge with dechallenge "unknown" needs clarification.

8. Data-quality findings requiring source verification

Finding	Cases	Comment
Indication not consistent with drug	001, 002, 003, 004, 006, 007, 008, 009, 010, 011	10 of 12. Verify against source; consider wrong-product or transcription error. Values retained as reported. Only 005 and 012 are plausible.
Implausibly high dose	004 (amlodipine 1,000 mg/day); 007 (omeprazole 1,000 mg/day)	Possible overdose or unit/transcription error; if overdose is confirmed, code it separately.
Sub-therapeutic or non-standard dose	001, 002, 003, 006, 008, 009, 010, 011	Amoxicillin 5-20 mg, atorvastatin 5 mg, metformin 5 mg, losartan 5 mg, sertraline 20 mg: check units and strength.
Hospitalisation needs clarification	003 (nausea), 010 (myalgia)	Obtain admission reason and dates; for 010 consider CK to exclude rhabdomyolysis (no diagnosis inferred).
Dechallenge/rechallenge inconsistencies	001, 002, 003, 004, 007, 008, 010, 011, 012	See Section 7.
Unsupported seriousness	All serious cases	No rationale supplied for "medically important"; admission details missing for 003 and 010.
Unlisted event	008	Unlisted in the synthetic CDS only; check the actual product label.
No dates, country or reporter identity	All	Day 0, reporting clocks and duplicate checks cannot be done.

9. Conclusion and limitations

All 12 cases were compiled into ICSR sections (patient, reporter, drug, reaction), mapped to MedDRA PT/SOC, and assessed for validity, seriousness, listedness, reportability and provisional causality. Values were transcribed as supplied and nothing was corrected silently. Causality categories are provisional pending the follow-up queries. MedDRA codes and version, and listedness against the actual product label, must be verified in the validated safety database before any regulatory use.

Role	Name	Signature	Date
Prepared by	Durga Chandra Kanth Samayam, Pharmacovigilance Intern, 13 october 2026	Durga Chandra Kanth Samayam	13/10/2026

Reviewed by (QC)	Pending review		

Abbreviations: CDS, core data sheet; CK, creatine kinase; ICSR, individual case safety report; LLT, lowest level term; PBRER/PSUR, periodic benefit-risk / safety update report; PT, preferred term; SOC, system organ class; WHO-UMC, World Health Organization Uppsala Monitoring Centre.